

**THE PRESS BOX & PYRAMID CONCESSION**

**Press Box:**

- ~200h
- ADD-ON ONLY
- PAR-1
- TIMI 50
- WIN: MI/STROKE
- LOSS: bleeding doubled
- INDICES PRIOR MI/PAD
- NOT NSTEMI
- stays weeks
- antidote PLATELETS
- STROKE/TIA
- PRIOR ICH
- >75 <=60kg
- NO DETAIL WORDS
- ? PLATELETT

**Pyramid Concession:**

- PYRAMIDS**
- PDE blocked**
- PDE adenos**
- rising cAMP**
- calm plates**
- arrows**
- AGGRENOL**
- simplified core names
- dipyridamole, aspirin
- visual dosing. TWICE DAILY
- AGGRENOL > ASA**
- PROFESS FAILED**
- 9.0% CLOPIDOGREL (1)**
- 8.8% CLOPIDOGREL (1)**
- INDICATIONS**
- TIA/STROKE
- ALWAYS AGGRENOL
- side effects**
- why clopidogrel**
- headache
- BID dosing
- CAD steal
- dizzy eyes
- queasy stomach
- low blood
- SE
- NO SYMPTOMATIC CAD**
- CORONARY STEAL + HYPOTENSION**
- into core labels
- into core
- NO SYMPT CAD STEAL HYPOTEN.**
- TREATS CLAUDICATION SYMPTOMS**
- CORONARY DILATE(2)**
- CORONARY STEAL(2)**
- unlike antiplatelets... CV EVENTS not symptoms
- visual of antiplatelet
- visual its symptoms
- Sign - total 2 data label

**Cilostazol:**

- CILOSTAZOL**
- CORONARY STEAL ACTIONS**

**Other:**

- VON WILLE BRAND FIELD
- Car Don't For It

**Total Press Box words: 19**

**Total PC: 1+0+3+4+6+0+0+4=18 words**

**Total Cilostazol: 1+4+2=7 words**



### 1. Clopidogrel ~200h clock

**WHAT YOU SEE**

Giant press-box clock reading '~200h'.

**WHAT IT ENCODES**

Clopidogrel is an irreversible thienopyridine prodrug; its antiplatelet effect lasts the platelet lifespan (~7–10 days) once steady state is reached. The 200h motif marks its long, irreversible offset versus the reversible agents.

**BOARD TRAP**

Clopidogrel binds P2Y12 irreversibly — recovery requires new platelets (~5–7 days hold pre-surgery), not drug clearance.

### 2. TIMI 50 scoreboard

**WHAT YOU SEE**

'TIMI 50' scoreboard with WIN (heart/brain) vs LOSS (bleeding doubled).

**WHAT IT ENCODES**

Trial-score motif: antithrombotic intensification cuts ischemic MI/stroke (the WIN) but roughly doubles major bleeding (the LOSS). The risk–benefit trade is the recurring board theme for adding/escalating antiplatelets.

**BOARD TRAP**

More platelet blockade = fewer ischemic events but more bleeding — always weigh both columns, never assume net benefit.

### 3. Clopidogrel 75 mg once daily

**WHAT YOU SEE**

'2.08mg... QD' / 75 mg once-daily dosing tag near the desk.

**WHAT IT ENCODES**

Clopidogrel maintenance is 75 mg once daily after a 300–600 mg load. Once-daily dosing reflects irreversible binding; it is the default add-on for secondary prevention and DAPT after ACS/PCI.

**BOARD TRAP**

Clopidogrel 75 mg QD vs ticagrelor 90 mg BID — once-daily dosing flags the irreversible thienopyridine.

### 4. CYP2C19 activation / poor metabolizers

**WHAT YOU SEE**

'why clopidogrel' / activation theme; prodrug needing conversion.

**WHAT IT ENCODES**

Clopidogrel is a prodrug requiring hepatic CYP2C19 to form its active metabolite. CYP2C19 loss-of-function carriers (poor metabolizers) get reduced antiplatelet effect and higher stent-thrombosis risk; PPIs like omeprazole inhibit CYP2C19 and blunt it.

**BOARD TRAP**

Omeprazole (CYP2C19 inhibitor) lowers clopidogrel activation — use pantoprazole; poor metabolizers should switch to ticagrelor/prasugrel.

### 5. Indices prior MI/PAD — not NSTEMI

**WHAT YOU SEE**

Placard 'INDICES PRIOR MI/PAD' and 'NOT NSTEMI', 'ADD-ON ONLY'.

**WHAT IT ENCODES**

Vorapaxar context: a PAR-1 thrombin-receptor antagonist indicated as ADD-ON for prior MI or PAD to reduce thrombotic events — NOT for acute NSTEMI and contraindicated with any stroke/TIA history (intracranial bleeding).

**BOARD TRAP**

Vorapaxar (PAR-1 antagonist) is add-on for stable prior-MI/PAD; it is contraindicated with prior stroke/TIA/ICH — never use in acute coronary syndromes alone.

### 6. PAR-1 receptor

**WHAT YOU SEE**

'PAR1' receptor tag at the lower press-box.

**WHAT IT ENCODES**

PAR-1 (protease-activated receptor-1) is the platelet thrombin receptor blocked by vorapaxar, adding a thrombin-mediated pathway block on top of COX/ADP inhibition.

**BOARD TRAP**

PAR-1 = vorapaxar's target (thrombin pathway); distinct from COX-1 (aspirin), P2Y12 (clopidogrel), PDE (dipyridamole/cilostazol).

### 7. Aggrenox — dipyridamole + aspirin

**WHAT YOU SEE**

'AGGRENOX' box: 'dipyridamole + aspirin', 'simplified core names', 'visual decoy', 'TWICE DAILY'.

**WHAT IT ENCODES**

Aggrenox = extended-release dipyridamole + low-dose aspirin, dosed twice daily, used for secondary stroke/TIA prevention. Dipyridamole inhibits PDE and adenosine reuptake, raising platelet cAMP to inhibit aggregation.

**BOARD TRAP**

Aggrenox = dipyridamole + ASPIRIN (not clopidogrel). It is for stroke/TIA secondary prevention, dosed BID — headache is the classic limiting side effect.

### 8. PDE blocked → rising cAMP

**WHAT YOU SEE**

Pyramid 'PDE blocked', 'PDE adenosine', 'rising cAMP', 'less platelet activation'.

**WHAT IT ENCODES**

Dipyridamole and cilostazol both inhibit phosphodiesterase, preventing cAMP breakdown. Elevated platelet cAMP suppresses activation/aggregation and causes vasodilation (basis of the coronary-steal and headache effects).

**BOARD TRAP**

PDE inhibition (raising cAMP) is the dipyridamole/cilostazol mechanism — different from aspirin's COX-1/TXA2 block and clopidogrel's P2Y12 block.



9. Aggrenox vs ASA 9.0%

WHAT YOU SEE

Scoreboard 'AGGRENOX > ASA 9.0%' (WIN over aspirin).

WHAT IT ENCODES

In secondary stroke prevention (ESPS-2 / PRoFESS context), extended-release dipyridamole + aspirin showed benefit over aspirin alone for recurrent stroke risk reduction (the 9.0% motif), though tolerability (headache) limits use.

BOARD TRAP

Aggrenox edges aspirin for recurrent stroke prevention — but never use it for coronary indications; it is a stroke/TIA agent.

10. Clopidogrel PRoFESS 8.8%

WHAT YOU SEE

'CLOPIDOGREL PRoFESS 8.8% FAILED' scoreboard.

WHAT IT ENCODES

PRoFESS trial: clopidogrel vs aspirin–dipyridamole for recurrent stroke were essentially equivalent (no superiority, ~8.8% event motif). Clopidogrel monotherapy is an acceptable alternative for stroke patients intolerant of Aggrenox.

BOARD TRAP

PRoFESS showed no winner between clopidogrel and Aggrenox for stroke — pick based on tolerability (headache vs cost), not efficacy.

11. Dipyridamole side effects — headache

WHAT YOU SEE

Side-effect row: headache, dizzy eyes, queasy stomach, low blood (hypotension).

WHAT IT ENCODES

Dipyridamole/Aggrenox side effects stem from vasodilation: headache (most common, dose-limiting), dizziness, GI upset, and hypotension/flushing. Headache often improves with continued use or slow titration.

BOARD TRAP

Headache is the #1 reason patients stop Aggrenox — counsel and titrate; it is vasodilatory, not a bleeding sign.

12. Cilostazol — claudication

WHAT YOU SEE

'CILOSTAZOL' box: king figure, 'treats claudication symptoms', 'coronary dilator'.

WHAT IT ENCODES

Cilostazol is a PDE3 inhibitor that raises cAMP, causing vasodilation and platelet inhibition. Its primary indication is intermittent claudication (PAD) — improves walking distance — not acute coronary syndromes.

BOARD TRAP

Cilostazol treats claudication (PAD walking distance); it is NOT a primary cardiac antiplatelet despite mild antiplatelet action.

13. Cilostazol contraindicated in CHF

WHAT YOU SEE

Red warning: 'NO symptomatic CAD — coronary steal + hypotension', 'CV events not significant'.

WHAT IT ENCODES

Cilostazol is CONTRAINDICATED in heart failure of any severity (PDE3 inhibitors increase mortality in HF, like milrinone). Also avoid in symptomatic CAD — vasodilation can trigger coronary steal and hypotension.

BOARD TRAP

Cilostazol is contraindicated in ANY heart failure — the single most-tested cilostazol fact (class effect of PDE3 inhibitors raising HF mortality).

14. No detail words / minimal CAD benefit

WHAT YOU SEE

Red note: 'NO DETAIL WORDS', 'NO SYMPT CAD', stop signs over CAD/coronary.

WHAT IT ENCODES

Reinforces that cilostazol's evidence base is for PAD/claudication symptom relief, not cardiovascular event reduction — and it must be avoided where coronary steal or HF could be precipitated.

BOARD TRAP

Don't extend cilostazol's PAD benefit to cardiac protection — it does not reduce major CV events and is unsafe in CAD/HF.

15. Word-count scoreboards

WHAT YOU SEE

Bottom tallies: 'Total Press Box words 19', 'Total PC ...18 words', 'Total Cilostazol 1+4+2=7 words'.

WHAT IT ENCODES

Reema's self-check tallies confirm each concession (press box = clopidogrel/vorapaxar, pyramid = Aggrenox/dipyridamole, cilostazol) has its core labels memorized — a study-completeness motif, not a drug fact.

BOARD TRAP

These counts are a memorization checklist, not exam content — the testable facts are the mechanisms and contraindications above.